**POSITION TITLE:** Nurse (Consultant)
**ORGANIZATIONAL TITLE:** Policy and Procedures Officer
**PP/SERIES/GRADE:** GS-0610-12
**ORG LOCATION:** DHHS, IHS, Phoenix Area, Phoenix Service Unit, Phoenix Indian Medical Center, Division of Quality Management, Performance Improvement
**FLSA STATUS:** Exempt
**JOB CODE:** PX3147

## INTRODUCTION

This position is organizationally located in the Regulatory Compliance and Accreditation Department under the Division of Quality Management at the Phoenix Indian Medical Center (PIMC). This position serves as the Policy and Procedures Officer for hospital wide policies covering inpatient, ambulatory care, emergency and specialty areas. Incumbent serves as an analyst and advisor with the professional knowledge of a registered nurse who provides consultative and advisory services for the development, content, and authentication of the organization's policies and procedures, clinical protocols, and clinical practice guidelines

PIMC is a 129-bed medical center, a hospital based outpatient clinic, and three satellite clinics. Together these facilities serve over 7,600 Native American members of the local Tribal communities surrounding the greater Phoenix metropolitan area. In addition, PIMC serves as a Regional Medical Center and Referral Center that covers a three-state region (Arizona, Nevada, and Utah.) Each Tribal community is a stakeholder in the level of health care being provided.

The medical center provides inpatient and comprehensive ambulatory care, emergency medical, dental, eye care, environmental health, community health nursing, social services, preventive and behavioral health, with emergency care on a 24-hour basis, 7 days a week. PIMC has over 67,000 active clinical users, with over 3000 inpatient admissions/year and over 400,000 outpatient visits/year.

## MAJOR DUTIES AND RESPONSIBILITIES

As the Policy and Procedures Officer, performs the following duties:

Analyzes organizational policies to assure that the content is valid, consistent, and without conflicting content assuring that the procedures describe actual or intended practice.

Stays abreast of changes in the various regulations and standards to develop new policies or revise current policies to ensure that the organization is compliant.

Develops and tracks new policies to meet the changing needs of the organization.

Oversees the coordination of the distribution of all new and revised policies.

Manages the on-line organizational policies and procedures.

Advises department managers and executive staff on ways to implement new regulations and processes.

Collaborates with management to maintain the various departments' scope of services.

Initiates, coordinates, and facilitates clinical and administrative multidisciplinary teams for the development and implementation of policies and procedures, clinical protocols, and clinical practice guidelines.

Research and advise on best practices to support organizational continual survey readiness

Provide guidance for developing and implementing competencies, internal logs and other forms of documentation that assist with meeting regulatory compliance and accreditation standards

Performs other related duties as assigned.

## FES FACTORS

### FACTOR 1 - KNOWLEDGE REQUIRED BY THE POSITON LEVEL 1-7

Professional knowledge and ability to apply a wide range of professional nursing/health care concepts, principles, and practices of advanced nature is required to sufficiently perform studies, advise management and develop policies and procedures, and carry out functions as the Policy and Procedures Officer.

Incumbent must possess and maintain a current, valid, and active state license/registration as a registered professional nurse.

Professional knowledge and expertise of a registered nurse to soundly apply knowledge and skill in a wide range of pertinent health care related regulations, such as the Joint Commission (TJC), Centers for Medicare & Medical Services (CMS), OSHA, Safety codes - State and Federal, clinical care standards and other applicable standards and guidelines for the medical facility. Additionally, detailed knowledge of the Service Unit, Phoenix Area, and IHS policy and procedures is required.

Knowledge in observing activities, processes, and demonstrations and then analyzing the information to develop policies for a complex health care environment.

Knowledge and skill in obtaining, analyzing, and selecting pertinent information, determining the most logical and effective sequence of the policy. Knowledge and skill in articulating complex information clearly, accurately, and thoroughly; apply consistent style, grammar and punctuation practices.

Knowledge and ability to consult with subject clinical and administrative experts to verify policy information.

Knowledge of medical literature search techniques.

Professional knowledge and expertise as a health care provider, performing clinical duties related to the nursing profession.

Broad medical knowledge and technical understanding of disease process, treatment standards of care and the comprehensive process of delivering health care services as described in the scope of TJC standards and chapters.

Knowledge and experience in medical staff organization and function and requirements.

Knowledge and skill in the use of word processing programs and inter and/or intranet.

### FACTOR 2 - SUPERVISORY CONTROLS LEVEL2-4

The supervisor outlines overall objectives and available resources. The employee and supervisor, in consultation, discuss timeframes; scope of the assignment; including possible stages, and possible approaches.

As an expert, the incumbent works independently to plan and complete work assignments. The incumbent determines the most appropriate principles, and methods to apply in all phases of assignments, including the approach to be taken, degree of intensity, and depth of research in management advisories; interprets regulations on his/her own initiative, applies new methods to resolve complex and/or intricate, controversial, or unprecedented issues and problems, and resolves most of the conflicts that arise; and keeps the supervisor informed of progress and of potentially controversial matters.

The supervisor reviews work for soundness of overall approach, effectiveness in meeting requirements or producing expected results, the feasibility of recommendations, and adherence to requirements. The supervisor does not usually review methods used.

**FACTOR 3 - GUIDELINES** **LEVEL 3-4**

A wide variety of reference materials and manuals, including PSU, PAO, and IHS policies/procedures, standards from The Joint Commission and other regulatory agencies (i.e., OSHA, CMS) are available, but are not completely applicable to the work or have gaps in specificity. Further guidance comes from knowledge of disease processes, health care services processes, and evolving clinical practice standards. The incumbent applies considerable judgement in interpreting, adapting/modifying, researching, choosing, developing, and applying available guidelines for adaptation to specific problems or issues. The employee analyzes results and recommends changes.

**FACTOR 4 - COMPLEXITY** **LEVEL4-4**

Work consists of a variety of duties requiring the application of many different and unrelated clinical and administrative processes and clinical systems analysis methods to a broad range of Pl activities or to activities or to the in-depth analysis of PI issues. The incumbent makes decisions that involve major uncertainties with regard to the most effective approach or methodology to be applied. These changes typically result from continuing changes in service unit requirements or rapidly evolving technology in the health care industry. The work requires originating new techniques, establishing criteria, or developing new information.

**FACTOR 5 - SCOPE AND EFFECT** **LEVELS-4**

Independently interfaces with medical, nursing, management, and other staff to provide effective and approach policies and procedures, as necessary, when deficiencies and systems problems, or variation from the standards and processes are found. The work involves establishing criteria; formulating projects; assessing program effectiveness; or investigating or analyzing a variety of unusual conditions, problems, or questions. Activities affect PSU wide activities and are essential for fulfilling mission; achieving quality and safe patient care; TJC compliance; and hospital continual survey readiness.

**FACTOR 6 - PERSONAL CONTACTS** **LEVEL6-3**

Contacts are with multidisciplinary (medical staff, nursing staff, executive administration, and non-medical staff of the hospital) and interagency staff (headquarters, the Arizona PRO, CMS, and other agencies and organizations), as appropriate; and other representatives from various regulatory entities outside the agency in a moderately unstructured setting. Contacts are generally made face to face or by telephone and professional conduct is expected.

**FACTOR 7 - PURPOSE OF CONTACTS** **LEVEL7-C**

Contacts are to plan, coordinate, or advise on PI efforts, or to resolve issues or operating problems by influencing or persuading people who are working toward mutual goals and have basically cooperative attitudes. Contacts typically involve formalizing processes by writing policies and procedures.

**FACTOR 8 - PHYSICAL DEMANDS** **LEVEL 8-1**

Work is sedentary. Some work may require walking and standing in conjunction with travel and to attendance at meetings and conferences away from the work site. Projects may demands the incumbent to work under stress, to meet short deadlines, or cope with changing priorities and multiple projects and tasks that are varied in nature. The computer work requires long periods of time using keyboard and a computer and CRT.

**FACTOR 9- WORK ENVIRONMENT** **LEVEL9-1**

Work area is adequately lighted, heated, and ventilated. The work environment involves everyday risks or discomforts that require normal safety precautions.

**OTHER SIGNIFICANT FACTORS**

The incumbent is required to maintain a current, valid, and state licensure, registration, and/or certification in his or her medical specialty.

Conditions of employment - this position requires, at a minimum, Level 5 - MBI moderate risk level since the contents of the PD meets the Public Trust position criteria.

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Status Old PolicyStat ID 16791609

| | | | |
|---|---|---|---|
| Origination | November 26th, 2019 | Owner | Clinton Pope: Chief Quality Officer |
| Last Approved | September 30th, 2024 | Area | Administrative - Leadership |
| Effective | September 30th, 2024 | Applicability | Phoenix Service Unit |
| Last Revised | September 30th, 2024 | | |
| Next Review | September 30th, 2027 | | |

# Policy Management

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A. **Purpose:** To continually improve the care, treatment, and services delivered in the Phoenix Service Unit (PSU) through standardized management practices for policies and operational documents.

B. **Scope:** This policy applies to all PSU policies and operational documents and to all PSU staff.

C. **References:**

1. The Joint Commission, Comprehensive Accreditation Manual for Hospitals, current edition
2. Indian Health Service (IHS), Indian Health Manual (IHM), 5-15 – Records Management Program
3. Phoenix Service Unit, Quality Management Policy

D. **Definitions:**

1. **Accreditation Standards:** Formal criteria and expectations established by accrediting organizations (e.g., The Joint Commission) that guide healthcare organizations in delivering safe, high-quality care and operations.
2. **Approver**: Individuals assigned to review a document for official approval prior to it being published using an electronic approval workflow.
3. **Document:** The term document is used to refer to PSU policies and operational documents (procedures, guidelines, plans, standing orders, committee charters, etc.).
4. **Document Owner**: The individual assigned with responsibilities to manage document updates, revisions, approvals, education, and implementation of the policy,


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procedures, practices, etc.

5. **Evidence-Based Practices:** Systematically developed clinical or operational practices based on the integration of the best available research evidence, clinical expertise, and patient values to support decision-making and improve outcomes.
6. **Guideline**: A general set of rules, principles, or standards to guide decisions/actions.
7. **Management**: Individuals with daily management responsibilities and accountabilities for projects, programs, departments, equipment, and service lines including all supervisors and leaders in the organization.
8. **Operational Continuity:** The uninterrupted functioning of healthcare services and adherence to standards during periods of change, policy transition, or review cycles to ensure patient safety and compliance.
9. **Plan**: A framework for change to accomplish objectives, addressing the scope, resources, and schedule to meet those objectives. A plan includes a risk assessment and mitigation activities, describes the responsibilities of individuals and groups, and provides measures of success.
10. **Policy**: Duly approved statements of intent to assist in subjective and objective decision making. Policy often contains guidelines, procedures, protocol, regulation, and law with ways to operationalize the intent within the organization. Policies are written specific to the organization to establish and improve standards appropriate to the current culture, maturity, and constraints of the organization. As the organization matures, policies are revised to improve standards and associated practices.
11. **Policy Site Administrator**: Individuals assigned administrative privileges and access within the electronic repository, responsible for setting user permissions and managing the repository to maintain accurate user lists, permission levels, and assist in monitoring adherence to document review cycles. Policy Site Administrators develop and maintain competency in the use of the electronic repository program and serve as Super Users to the organization. Policy Site Administrators assist with technical issues according to ability, and refer all other issues to the vendor support desk. The PSU IT Department does not provide support for the electronic repository. Policy Site Administrators assist in providing and obtaining training for new and current users. Policy Site Administrator designation is determined by policy program leaders.
12. **Procedure**: Step-by-step actions, often with a specific sequence, to ensure reliable outcomes, reduce variation, and mitigate risk.
13. **Stakeholder**: Individuals affected by or with interest in a specific document, including care providers, committee members, staff members, management, patients, and community members. Stakeholders may review content and recommend changes.
14. **System of Continual Improvement**: A culture of non-punitive strategies to promote open discussion about issues, concerns, deviations, and risks following principles of psychological safety and a just culture; and the pursuit of excellence by iteratively improving the following major organizational factors over time: 1) Standard work,

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operational designs and plans as documented in policy and operational documents; 2) Healthcare operations, day-to-day activities, interactions, structures, and constraints of the healthcare system; 3) Perceptions of Care, perspectives and paradigms of patients, staff, partners, and community members; and, 4) Strategic alignment to organizational mission, vision, values, and goals.

15. **Regulatory Requirements:** Rules, laws, and directives issued by federal, state, or accrediting agencies that healthcare organizations must comply with to maintain licensure, certification, or accreditation.

E. **Policy**

1. PSU documents, as defined in this policy, must be kept in the electronic document repository and managed and approved as set forth in this policy.
2. PSU documents must be managed and standardized to promote a system of continual improvement of the care, treatment, and services delivered.
3. Documents will have established dates for review and must be reviewed promptly to incorporate revisions.
    a. PSU documents must be reviewed, revised, and approved at least every three years. However, unless otherwise stated, all approved documents shall remain in full effect beyond their scheduled review date until such time as they are formally reviewed and either rescinded, revised, or superseded. The presence of a lapsed review date shall not, in itself, invalidate the document or exempt staff from adherence. All personnel are expected to comply with the most current version of each document unless notified otherwise by authorized leadership.
    b. Policy Owners are responsible for initiating timely reviews and updates; however, operational continuity and patient safety require that existing policies remain enforceable to prevent procedural gaps.
    c. Standing orders expire one year from being signed, unless a shorter time is indicated.
    d. Documented plans are reviewed, revised, and approved annually, unless explicitly stated within the document that another cycle is being used. PSU mandates continued use of and adherence to PSU plans beyond their review date until the document is rescinded, recertified, or superseded
4. Stakeholders must be informed, trained, and competent to comply with PSU documents.
5. Acronyms and abbreviations are authorized for use within PSU documents. They must be clearly spelled out in full the first time they are used. Avoid documents with too many acronyms or abbreviations, which make it difficult to read. Avoid acronyms and abbreviations in document titles.
6. All new and revised documents must be approved through established approval workflows within the electronic repository. Approval workflows will include:
    a. Document owner

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b. Applicable subject matter experts

c. Applicable Quality Division experts

d. Applicable Committees / Governance

e. Senior leader in the document owner's chain of command

f. Policy & Procedure Nurse Consultant

g. Superseded documents are maintained on file in accordance with the Retention of Records policy

F. **Roles & Responsibilities:**

1. **Document Owners:**

a. Policy Owners are responsible for initiating and completing timely reviews of assigned policies according to the established review cycle (e.g., every 1–3 years or as designated). In addition: Ensure documents are managed with timely updates, revisions, approvals, education, and implementation according to the assigned review cycle.

b. Policy Owners must actively monitor for changes in evidence-based practices, clinical guidelines, regulatory requirements, and accreditation standards relevant to their policy content.
When significant updates in clinical evidence or practice standards occur, policies must be reviewed and updated as soon as reasonably possible, regardless of the next scheduled review date.

c. Delays in policy updates must be documented, including justification and anticipated timelines, and escalated to the appropriate committee or leadership body if necessary.

d. Ensure standing orders are renewed prior to expiration, and if a standing order does expire, coordinate with management to ensure staff are aware and do not utilize the expired standing order

e. Ensure content of documents are accurate and comply with current accreditation standards, higher-headquarter policy within the IHS and federal government, regulations, laws, and evidence-based best practices and guidelines, which may require document revisions prior to assigned review cycle

f. Ensure stakeholders, including appropriate committees, are identified and given opportunity to review new and revised documents for input and recommendations

g. Review other PSU documents to avoid conflicting and duplicate information

h. Ensure documents are written using approved templates

i. Ensure staff are informed, trained, and competent when implementing new and revised documents according to the following Document Impact Assessment:

i. Requires education of content in staff meetings and unit or

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department communications if at least one of the following apply:

a. The document requires awareness of new or revised content

b. The document requires a change in behavior or workflow to comply with new or revised non-regulatory content

ii. Applicable staff must receive training (in-person or computer-based learning) with documentation of participation and applicable staff must sign an attestation that the document was reviewed if at least one of the following apply:

a. The document requires a significant change in behavior or workflow to comply with new or revised content

b. An accreditation standard or a regulation requires documentation of education

iii. Applicable staff must receive training (in-person or computer-based learning) with documentation of participation and must either receive a passing score on a post test or demonstrate competency to an appropriate preceptor if at least one of the following apply.

a. The document requires development and mastery of new skills to comply with new or revised content

b. Failure to adhere to new or revised content could result in harm to staff, patients, visitors, or organizational reputation

c. The document changes knowledge or skills involved in a nursing practice or procedure that is considered low-volume-high-risk (requires competency validation)

j. Coordinate with Staff Development, Nursing Education, and management to implement education requirements

k. Ensure education requirements are fully completed within 90 days of the document being published

l. Notify the Policy Site Administrator of any documents that need to be retired

m. Actively monitor for updates in evidence-based practices, clinical guidelines, regulatory requirements, and accreditation standards relevant to the content of assigned documents. If significant changes occur, the Document Owner must review and update the policy as soon as reasonably possible, even if the scheduled review date has not yet arrived. Delays must be documented, justified, and elevated to the appropriate leadership body as needed.

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2. **Management:**
    a. Ensure PSU Documents promote effective and reliable operations in compliance with applicable accreditation standards, regulations, laws, statutes, and evidence-based best practices and guidelines
    b. Ensure document owners fulfill their duties and responsibilities.
    c. Ensure that appropriate resources and accountability mechanisms are in place to maintain policy relevance and regulatory alignment.
    d. Ensure standing orders are managed, utilized appropriately by staff, and that staff are aware of and do not utilize expired standing orders
    e. Select appropriate document owners, which is typically a manager with appropriate expertise and accountability for ongoing compliance to the document
    f. Assess compliance to PSU documents by observing for deviations in practices, effectiveness, staff knowledge, and adverse outcomes
    g. Leadership shall monitor and prioritize overdue policy reviews to ensure alignment with current regulatory and clinical standards
    h. When gaps exist between PSU documents and operational practices or staff knowledge, or when there are adverse outcomes or safety concerns, management investigate underlying causes, iteratively improve practices, and revise PSU documents to ensure high reliability of care, treatment, and services
    i. Ensure new employees, contractors, students, and volunteers are oriented to key documents during onboarding and orientation to the PSU and to their department
    j. When new or revised PSU documents are published, ensure stakeholders receive awareness, training, education, and competency assessments appropriate to operationalize the intent of the document to ensure compliance according to the Document Impact Assessment
    k. For high-risk and problem-prone processes and systems, ensure staff receive competency assessments to validate their understanding of and compliance to standardized practices
3. **Stakeholders:**
    a. Ensure personal competency of assigned duties, tasks, roles, and responsibilities
    b. Review and understand applicable PSU documents
    c. Complete all annual mandatory training
    d. Comply with PSU documents and promote high-quality and safe delivery of care, treatment, and services
    e. When gaps exist between PSU documents and operational practices or staff knowledge, or when there are adverse outcomes or safety concerns, report findings to management and report safety concerns or events in the

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IHS Safety Tracking and Reporting (I-STAR) system

f. Provide meaningful feedback and recommendations to the document owner when reviewing new and revised documents

4. **Policy & Procedure Nurse Consultant:**

a. Ensures PSU documents support operational needs and the IHS mission by:

i. Engaging with and assisting document owners, management, and stakeholders in collaborative and supportive ways

ii. Advising on how to implement regulations, standards, and evidence-based practices to support and improve patient and staff safety

iii. Assisting to develop new and revised documents to meet changing needs and address quality and safety concerns and issues

iv. Initiating, coordinating, and facilitating multidisciplinary teams to develop and implement PSU documents for clinical and administrative needs

v. Assisting with the development and implementation of competency assessments, tracking logs, and other forms of documentation to meet educational requirements according to policy and accrediting/regulatory bodies

vi. Overseeing the coordination and distribution of new and revised documents

vii. Serving as the Policy Site Administrator, managing the electronic document repository including accounts, permissions, approval workflow creation and edits, document ownership, etc.

viii. Observing operational activities, processes, and demonstrations to assess compliance to PSU documents, assisting with on-the-spot corrections in collaborative and supportive ways, reporting findings to management, and assisting with improvement efforts

b. Ensures new and revised documents are reviewed for:

i. Alignment with accreditation standards, regulatory requirements, and evidence-based practices and guidelines

ii. Comparison with existing documents to identify inconsistencies, conflicting information, and duplicate information

iii. Standardized formatting

iv. shall monitor policy review compliance across departments, identify overdue policies, and support prioritization of high-risk areas.

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v. *Note: When reviewing documents outside the Nurse Consultant's scope of expertise, subject matter experts are consulted to assist with the review. The Nurse Consultant does not provide legal advice or opinions, only attorneys in the Office of the General Counsel are authorized to provide legal advice or issue legal opinions.*

c. Coordinates revision of PSU documents by:
   i. Sending email reminders to document owners and their supervisor at 60, and 30 days prior to the document's due date for review
   ii. Sending email notifications to document owners, their supervisor, and senior leaders when documents have not been renewed by the due date for review
   iii. Sending email notifications to document owners, their supervisor, and senior leaders when standing orders are at 30 days prior to expiring and again if the standing order does expire
   iv. Engaging with document owners, management, and stakeholders to understand and assist with removing barriers to renewing documents
   v. Initiating, coordinating, and facilitating multidisciplinary teams to assist with renewing documents

d. Collects, analyzes, and reports relevant statistics to Quality Council highlighting opportunities to improve document management and compliance

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## All Revision Dates

September 30th, 2024, January 31st, 2023, November 26th, 2019

## Approval Signatures

| Step Description | Approver | Date |
|---|---|---|
| Policy Officer | Kevin Turnage: Policy and Procedure Officer, Nurse Consultant | September 30th, 2024 |
| Executive Committee | Debra Ward Lund: CEO | September 27th, 2024 |
| Chief Quality Officer | Clinton Pope: Chief Quality Officer | September 26th, 2024 |


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| | |
|---|---|
| Clinton Pope: Chief Quality Officer | September 26th, 2024 |

## Applicability

Phoenix Service Unit

## History

**Edited by Pope, Clinton: Chief Quality Officer** on 9/26/2024, 6:12PM EDT

> Major rewrite to align with other policy and to clarify roles and responsibilities

**Last Approved by Pope, Clinton: Chief Quality Officer** on 9/26/2024, 6:12PM EDT

**Last Approved by Pope, Clinton: Chief Quality Officer** on 9/26/2024, 6:13PM EDT

**Last Approved by Ward Lund, Debra: CEO** on 9/27/2024, 7:06PM EDT

**Last Approved by Turnage, Kevin: Policy and Procedure Officer, Nurse Consultant** on 9/30/2024, 12:31PM EDT

> It should be noted that the Policy and Procedure Nurse Consultant has raised concerns regarding the release of this policy in its current form, highlighting the need for improvements.

**Activated** on 9/30/2024, 12:31PM EDT

**Administrator override by Pope, Clinton: Chief Quality Officer** on 1/14/2025, 2:16PM EST

> For the Policy & Procedure Nurse Consultant: Notifications for standing orders changed from 20 days prior to expiration to 30 days prior to expiration and additionally notifications sent if the standing order expires.

**Administrator override by Pope, Clinton: Chief Quality Officer** on 3/25/2025, 3:13PM EDT

> Clarified that PSU mandates continued use of and adherence to PSU documents beyond their review date until the document is rescinded, recertified, or superseded.

**Administrator override by Turnage, Kevin: Policy and Procedure Officer, Nurse Consultant** on 4/2/2025, 1:18PM EDT

> Updated Definitions


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**Administrator override by Pope, Clinton: Chief Quality Officer** on 5/13/2025, 12:23PM EDT

> In a few places, changed "policy" to "document" to be more inclusive of the various documents this policy applies to.

**Administrator override by Pope, Clinton: Chief Quality Officer** on 5/27/2025, 11:26AM EDT

> Added statement "The Nurse Consultant does not provide legal advice or opinions, only attorneys in the Office of the General Counsel are authorized to provide legal advice or issue legal opinions."

**Draft saved by Ward Lund, Debra: CEO** on 6/25/2025, 7:50PM EDT

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Status **Active** PolicyStat ID **18430120**

| | | | |
|---|---|---|---|
| Origination | November 26th, 2019 | Owner | Clinton Pope: Chief Quality Officer |
| Last Approved | June 26th, 2025 | Area | Administrative - Leadership |
| Effective | June 26th, 2025 | Applicability | Phoenix Service Unit |
| Last Revised | June 26th, 2025 | | |
| Next Review | June 25th, 2028 | | |

# Policy Management

COPY

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12. **Procedure**: Step-by-step actions, often with a specific sequence, to ensure reliable outcomes, reduce variation, and mitigate risk.
13. **Stakeholder**: Individuals affected by or with interest in a specific document, including care providers, committee members, staff members, management, patients, and community members. Stakeholders may review content and recommend changes.
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operational designs and plans as documented in policy and operational documents; 2) Healthcare operations, day-to-day activities, interactions, structures, and constraints of the healthcare system; 3) Perceptions of Care, perspectives and paradigms of patients, staff, partners, and community members; and, 4) Strategic alignment to organizational mission, vision, values, and goals.

15. **Regulatory Requirements:** Rules, laws, and directives issued by federal, state, or accrediting agencies that healthcare organizations must comply with to maintain licensure, certification, or accreditation.

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3. Documents will have established dates for review and must be reviewed promptly to incorporate revisions.
    a. PSU documents must be reviewed, revised, and approved at least every three years. However, unless otherwise stated, all approved documents shall remain in full effect beyond their scheduled review date until such time as they are formally reviewed and either rescinded, revised, or superseded. The presence of a lapsed review date shall not, in itself, invalidate the document or exempt staff from adherence. All personnel are expected to comply with the most current version of each document unless notified otherwise by authorized leadership.
    b. Policy Owners are responsible for initiating timely reviews and updates; however, operational continuity and patient safety require that existing policies remain enforceable to prevent procedural gaps.
    c. Standing orders expire one year from being signed, unless a shorter time is indicated.
    d. Documented plans are reviewed, revised, and approved annually, unless explicitly stated within the document that another cycle is being used. PSU mandates continued use of and adherence to PSU plans beyond their review date until the document is rescinded, recertified, or superseded
4. Stakeholders must be informed, trained, and competent to comply with PSU documents.
5. Acronyms and abbreviations are authorized for use within PSU documents. They must be clearly spelled out in full the first time they are used. Avoid documents with too many acronyms or abbreviations, which make it difficult to read. Avoid acronyms and abbreviations in document titles.
6. All new and revised documents must be approved through established approval workflows within the electronic repository. As appropriate, approval workflows will include:

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a. Document owner
b. Applicable subject matter experts
c. Applicable Quality Division experts
d. Applicable Committees / Governance
e. Senior leader in the document owner's chain of command

7. Superseded documents are maintained on file in accordance with the Retention of Records policy

F. **Roles & Responsibilities:**

1. **Document Owners:**

a. Policy Owners are responsible for initiating and completing timely reviews of assigned policies according to the established review cycle (e.g., every 1–3 years or as designated). In addition: Ensure documents are managed with timely updates, revisions, approvals, education, and implementation according to the assigned review cycle.

b. Policy Owners must actively monitor for changes in evidence-based practices, clinical guidelines, regulatory requirements, and accreditation standards relevant to their policy content.
When significant updates in clinical evidence or practice standards occur, policies must be reviewed and updated as soon as reasonably possible, regardless of the next scheduled review date.

c. Delays in policy updates must be documented, including justification and anticipated timelines, and escalated to the appropriate committee or leadership body if necessary.

d. Ensure standing orders are renewed prior to expiration, and if a standing order does expire, coordinate with management to ensure staff are aware and do not utilize the expired standing order

e. Ensure content of documents are accurate and comply with current accreditation standards, higher-headquarter policy within the IHS and federal government, regulations, laws, and evidence-based best practices and guidelines, which may require document revisions prior to assigned review cycle

f. Ensure stakeholders, including appropriate committees, are identified and given opportunity to review new and revised documents for input and recommendations

g. Review other PSU documents to avoid conflicting and duplicate information

h. Ensure documents are written using approved templates

i. Ensure staff are informed, trained, and competent when implementing new and revised documents according to the following Document Impact Assessment:

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i. Requires education of content in staff meetings and unit or department communications if at least one of the following apply:
    a. The document requires awareness of new or revised content
    b. The document requires a change in behavior or workflow to comply with new or revised non-regulatory content

ii. Applicable staff must receive training (in-person or computer-based learning) with documentation of participation and applicable staff must sign an attestation that the document was reviewed if at least one of the following apply:
    a. The document requires a significant change in behavior or workflow to comply with new or revised content
    b. An accreditation standard or a regulation requires documentation of education

iii. Applicable staff must receive training (in-person or computer-based learning) with documentation of participation and must either receive a passing score on a post test or demonstrate competency to an appropriate preceptor if at least one of the following apply.
    a. The document requires development and mastery of new skills to comply with new or revised content
    b. Failure to adhere to new or revised content could result in harm to staff, patients, visitors, or organizational reputation
    c. The document changes knowledge or skills involved in a nursing practice or procedure that is considered low-volume-high-risk (requires competency validation)

j. Coordinate with Staff Development, Nursing Education, and management to implement education requirements

k. Ensure education requirements are fully completed within 90 days of the document being published

l. Notify the Policy Site Administrator of any documents that need to be retired

m. Actively monitor for updates in evidence-based practices, clinical guidelines, regulatory requirements, and accreditation standards relevant to the content of assigned documents. If significant changes occur, the Document Owner must review and update the policy as soon as reasonably possible, even if the scheduled review date has not yet arrived. Delays must be documented, justified, and elevated to the appropriate

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leadership body as needed.

2. **Management:**
    a. Ensure PSU Documents promote effective and reliable operations in compliance with applicable accreditation standards, regulations, laws, statutes, and evidence-based best practices and guidelines
    b. Ensure document owners fulfill their duties and responsibilities.
    c. Ensure that appropriate resources and accountability mechanisms are in place to maintain policy relevance and regulatory alignment.
    d. Ensure standing orders are managed, utilized appropriately by staff, and that staff are aware of and do not utilize expired standing orders
    e. Select appropriate document owners, which is typically a manager with appropriate expertise and accountability for ongoing compliance to the document
    f. Assess compliance to PSU documents by observing for deviations in practices, effectiveness, staff knowledge, and adverse outcomes
    g. Leadership shall monitor and prioritize overdue policy reviews to ensure alignment with current regulatory and clinical standards
    h. When gaps exist between PSU documents and operational practices or staff knowledge, or when there are adverse outcomes or safety concerns, management investigate underlying causes, iteratively improve practices, and revise PSU documents to ensure high reliability of care, treatment, and services
    i. Ensure new employees, contractors, students, and volunteers are oriented to key documents during onboarding and orientation to the PSU and to their department
    j. When new or revised PSU documents are published, ensure stakeholders receive awareness, training, education, and competency assessments appropriate to operationalize the intent of the document to ensure compliance according to the Document Impact Assessment
    k. For high-risk and problem-prone processes and systems, ensure staff receive competency assessments to validate their understanding of and compliance to standardized practices
3. **Stakeholders:**
    a. Ensure personal competency of assigned duties, tasks, roles, and responsibilities
    b. Review and understand applicable PSU documents
    c. Complete all annual mandatory training
    d. Comply with PSU documents and promote high-quality and safe delivery of care, treatment, and services
    e. When gaps exist between PSU documents and operational practices or

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staff knowledge, or when there are adverse outcomes or safety concerns, report findings to management and report safety concerns or events in the IHS Safety Tracking and Reporting (I-STAR) system

f. Provide meaningful feedback and recommendations to the document owner when reviewing new and revised documents

4. **Policy & Procedure Nurse Consultant:**

a. Ensures PSU documents support operational needs and the IHS mission by:

i. Engaging with and assisting document owners, management, and stakeholders in collaborative and supportive ways

ii. Advising on how to implement regulations, standards, and evidence-based practices to support and improve patient and staff safety

iii. Assisting to develop new and revised documents to meet changing needs and address quality and safety concerns and issues

iv. Initiating, coordinating, and facilitating multidisciplinary teams to develop and implement PSU documents for clinical and administrative needs

v. Assisting with the development and implementation of competency assessments, tracking logs, and other forms of documentation to meet educational requirements according to policy and accrediting/regulatory bodies

vi. Overseeing the coordination and distribution of new and revised documents

vii. Serving as the Policy Site Administrator, managing the electronic document repository including accounts, permissions, approval workflow creation and edits, document ownership, etc.

viii. Observing operational activities, processes, and demonstrations to assess compliance to PSU documents, assisting with on-the-spot corrections in collaborative and supportive ways, reporting findings to management, and assisting with improvement efforts

b. Ensures new and revised documents are reviewed for:

i. Alignment with accreditation standards, regulatory requirements, and evidence-based practices and guidelines

ii. Comparison with existing documents to identify inconsistencies, conflicting information, and duplicate information

iii. Standardized formatting

iv. shall monitor policy review compliance across departments, identify overdue policies, and support prioritization of high-risk

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areas.

v. *Note: When reviewing documents outside the Nurse Consultant's scope of expertise, subject matter experts are consulted to assist with the review. The Nurse Consultant does not provide legal advice or opinions, only attorneys in the Office of the General Counsel are authorized to provide legal advice or issue legal opinions.*

c. Coordinates revision of PSU documents by:

i. Sending email reminders to document owners and their supervisor at 60, and 30 days prior to the document's due date for review

ii. Sending email notifications to document owners, their supervisor, and senior leaders when documents have not been renewed by the due date for review

iii. Sending email notifications to document owners, their supervisor, and senior leaders when standing orders are at 30 days prior to expiring and again if the standing order does expire

iv. Engaging with document owners, management, and stakeholders to understand and assist with removing barriers to renewing documents

v. Initiating, coordinating, and facilitating multidisciplinary teams to assist with renewing documents

d. Collects, analyzes, and reports relevant statistics to Quality Council highlighting opportunities to improve document management and compliance

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## All Revision Dates

June 26th, 2025, September 30th, 2024, January 31st, 2023, November 26th, 2019

## Approval Signatures

| Step Description | Approver | Date |
|---|---|---|
| Executive Committee | Debra Ward Lund: CEO | June 26th, 2025 |
| Chief Quality Officer | Clinton Pope: Chief Quality Officer | June 26th, 2025 |
| | Clinton Pope: Chief Quality Officer | June 26th, 2025 |


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## Applicability

Phoenix Service Unit

## History

**Edited by Ward Lund, Debra: CEO** on 6/25/2025, 7:52PM EDT

> Removed Policy & Procedure Nurse Consultant from approval workflow listed in Section E.6

**Draft saved by Pope, Clinton: Chief Quality Officer** on 6/26/2025, 11:57AM EDT

**Edited by Pope, Clinton: Chief Quality Officer** on 6/26/2025, 11:59AM EDT

> Minor adjustments to E. 6. & 7.

**Last Approved by Pope, Clinton: Chief Quality Officer** on 6/26/2025, 11:59AM EDT

**Last Approved by Pope, Clinton: Chief Quality Officer** on 6/26/2025, 11:59AM EDT

**Last Approved by Ward Lund, Debra: CEO** on 6/26/2025, 12:34PM EDT

**Activated** on 6/26/2025, 12:34PM EDT

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# Exhibit D – Workflow Comparison Chart (2024 vs. 2025)

**OSC Case No.: MA-25-004957 (Turnage, Health and Human Services)**

**Complainant:** Kevin D. Turnage

**Agency:** HHS / IHS / Phoenix Indian Medical Center

**Approval Workflow – Sept. 30, 2024 Policy (Old Policy)**

**Document Owner:** CQO Clinton Pope

1. Document Owner
2. Subject Matter Experts (SMEs)
3. Quality Division
4. Committees/Governance
5. Senior Leadership
6. **Policy & Procedure Nurse Consultant (Policy Officer – Kevin Turnage) ✓**
7. Executive Committee / CEO

**Approval Workflow – June 26, 2025 Policy (Current Policy)**

**Document Owner:** CQO Clinton Pope

1. Document Owner
2. Subject Matter Experts (SMEs)
3. Quality Division
4. Committees/Governance
5. Senior Leadership

6. **Policy & Procedure Nurse Consultant (Policy Officer)** ❌ *Removed*
7. Executive Committee / CEO

**Key Change**

- In 2024, the **Policy Officer** (Kevin Turnage) was explicitly required to approve policies.
- In 2025, authored by the same official (CQO Pope), the **Policy Officer was removed from the workflow**, stripping approval authority.

**Significance**

- The Policy Officer's approval authority was **formally granted by management** in 2024, not self-assigned.
- The same management official then deliberately **removed this authority** in 2025, materially diminishing the position's core duties.
- This change directly conflicted with the PD requirement to *"develop new policies or revise current policies to ensure that the organization is compliant."*

**Exhibit E – Timeline of Policy Changes (2019–2025)**

| Date / Version | Policy Officer Role | Approval Authority |
|---|---|---|
| **2019 – Origination (Nov. 26, 2019)** | Advisory on standards; policy development support; compliance observation; repository admin. | **Yes – approver included in workflow** |
| **2023 – Revision (Jan. 31, 2023)** | Role carried forward without change. | **Yes – approval authority retained** |
| **Sept. 30, 2024 – Old Policy** | Duties expanded: advising on regulations, facilitating teams, competencies/logs, overdue tracking, compliance observation. | **Yes – explicitly listed approver: "Policy Officer Kevin Turnage, Nurse Consultant"** |
| **June 26, 2025 – Current Policy** | Retains advisory/administrative tasks (assisting, coordinating, repository management, observing compliance). | **No – Policy Officer removed from approval workflow** |

**Key Change:** From Sept. 2024 through June 2025, I was formally a required approver. The June 2025 revision deliberately stripped this approval authority, directly conflicting with my Position Description.

**Exhibit F – Declaration of Kevin D. Turnage**

I, **Kevin D. Turnage**, declare under penalty of perjury as follows:

1. I am employed as a Nurse (Consultant), Policy & Procedures Officer (GS-12) at Phoenix Indian Medical Center.
2. My Position Description requires me to *"develop new policies or revise current policies to ensure that the organization is compliant."* Compliance authority is central to my role.
3. From **Sept. 30, 2024 through June 26, 2025**, the Policy Management Policy listed me as a required approver. The Approval Signatures block identified me as "Policy Officer Kevin Turnage, Nurse Consultant."
4. On **June 26, 2025**, a revised Policy Management Policy removed the Policy Officer from the approval workflow. I was no longer able to approve or reject policies.
5. Without approval authority, I cannot fulfill my PD's compliance mandate. My duties were reduced to nominal, advisory, or clerical tasks.
6. This change stripped me of core responsibilities, leaving my position hollowed out while my grade and title remained the same.
7. The removal of my duties occurred shortly after my May 21, 2025 protected disclosures.

I declare under penalty of perjury that the foregoing is true and correct.

Executed on: September 24, 2025
Gilbert, AZ

**/s/ Kevin D. Turnage**

**Kevin D. Turnage**

**Exhibit G – Supplemental Filing Memorandum**

**Re: OSC Case No. MA-25-004957 (Turnage, HHS/IHS)**

**Purpose**

This memorandum supplements my May 28, 2025 OSC complaint to add allegations of **constructive demotion** and **investigative leave violations**.

**1. Constructive Demotion**

- PD requires me to ensure compliance.
- Sept. 30, 2024 Policy Management Policy (authored by CQO Clinton Pope) expressly listed me as approver.
- June 26, 2025 revision (also by CQO Pope) removed me from the workflow.
- This stripped me of core duties, conflicting with my PD and constituting a constructive demotion under *Lovshin* and *Russell*.
- The timing closely followed my May 21, 2025 disclosures, showing retaliation.

**2. Investigative Leave Violation**

- I was placed on investigative leave exceeding 30 days, in violation of **5 U.S.C. § 6329b**.
- Required written approvals and reviews were not obtained.
- This was not in accordance with law, constituting a PPP under **§ 2302(b)(12)**.
- It also served as reprisal under **§ 2302(b)(8)**.

**3. Requested OSC Action**

- Investigate constructive demotion and leave violations.
- Consolidate with original May 28, 2025 PPP complaint.
- Corrective action: restore my approval authority, ensure compliance responsibilities match my PD, correct personnel record, and consider discipline against officials.

Respectfully,

**Kevin D. Turnage**

**Exhibit H – Executive Summary (1-Page)**

**Complainant:** Kevin D. Turnage

**Agency:** HHS / IHS / Phoenix Indian Medical Center

**OSC Case No.: MA-25-004957 (Turnage, Health and Human Services)**

**Key Facts**

- **Position Description (PX3147):** Requires me to *"develop new policies or revise current policies to ensure that the organization is compliant."* Compliance enforcement is the **core duty** of my role.
- **Sept. 30, 2024 Policy Management Policy:** Authored by CQO Clinton Pope. Lists me ("Policy Officer Kevin Turnage, Nurse Consultant") as a **required approver** in the Approval Signatures block.
- **June 26, 2025 Policy Revision:** Also authored by CQO Pope. **Removed the Policy Officer from the approval workflow**. Left me with only advisory/administrative tasks, eliminating approval authority necessary to enforce compliance.
- **Result:** My core duties were stripped, directly conflicting with my PD. This constitutes a **constructive demotion** under *Lovshin v. Navy* and *Russell v. Navy*.
- **Investigative Leave:** I was placed on investigative leave for longer than the 30 days allowed by **5 U.S.C. § 6329b**. No written extensions or required reviews were provided.
- **Protected Disclosures:** Prior to these actions, I disclosed failures to comply with federal standards, accreditation risks, and patient safety issues (May 21, 2025 memorandum).
- **Temporal Connection:** Removal of my duties occurred **within weeks** of my disclosures, showing a retaliatory motive.

**PPP Categories Alleged**

- **§ 2302(b)(8):** Retaliation for whistleblowing.
- **§ 2302(b)(9):** Retaliation for exercising appeal rights and protected activity.
- **§ 2302(b)(12):** Personnel actions not in accordance with law (investigative leave exceeding statutory limits).
- **§ 2302(b)(6):** Granting unauthorized advantage by stripping my independent compliance role and consolidating authority in management.

**Relief Requested**

- Restore my **approval authority and compliance duties** consistent with my Position Description.
- Correct personnel records to remove retaliatory actions.
- OSC investigation into the Agency's procedural violations and retaliatory conduct.
- Appropriate disciplinary action against responsible officials.

**/s/ Kevin D. Turnage**

Date: [Insert]

**Supplemental Filing – OSC PPP Complaint**

**Case No.: MA-25-004957 (Turnage, Health and Human Services)**

**Complainant:** Kevin D. Turnage

**Agency:** Department of Health and Human Services, Indian Health Service (IHS), Phoenix Indian Medical Center (PIMC)

**Purpose of Supplemental Filing**

This memorandum supplements my **May 28, 2025 OSC complaint** to add additional allegations of **Prohibited Personnel Practices (PPPs)** under **5 U.S.C. § 2302(b)**, arising from the same pattern of retaliatory conduct by Agency officials.

**1. Constructive Demotion – June 26, 2025 Policy Revision**

- My Position Description (PX3147) requires me to *"develop new policies or revise current policies to ensure that the organization is compliant."*
- From **September 30, 2024 through June 26, 2025**, the **Policy Management Policy** (authored by CQO Clinton Pope) expressly listed me as a required **approver** of PSU policies. The Approval Signatures block identifies "**Policy Officer Kevin Turnage, Nurse Consultant**," confirming my approval authority.
- On **June 26, 2025**, CQO Pope issued a new Policy Management Policy revision that **removed the Policy & Procedure Nurse Consultant (Policy Officer)** from the approval workflow.
- The removal of approval authority **stripped me of the core duties of my position** and made it impossible to fulfill the compliance responsibilities mandated by my PD.

- Under *Lovshin v. Navy*, 767 F.2d 826 (Fed. Cir. 1985), and *Russell v. Navy*, 6 M.S.P.R. 698 (1981), this constitutes a **constructive demotion**, which is an appealable adverse action.
- Because this removal of duties occurred shortly after my **May 21, 2025 protected disclosures**, it also constitutes **reprisal** in violation of **5 U.S.C. § 2302(b)(8) and (b)(9)**.

**2. Violation of Investigative Leave Statute – 5 U.S.C. § 6329b**

- The Agency placed me on investigative leave for a period **exceeding the 30-day statutory limit** under 5 U.S.C. § 6329b.
- Under the statute, any extension of investigative leave beyond 30 days requires:
    1. Written approval by the agency head or Chief Human Capital Officer; and
    2. Documentation of necessity, reviewed every 30 days.
- In my case, these statutory requirements were not met. No such written approvals or periodic reviews were provided.
- As a result, my extended enforced leave was **not in accordance with law, rule, or regulation**, constituting a PPP under **§ 2302(b)(12)**.
- The enforced leave also served as retaliation for my disclosures regarding patient safety and compliance deficiencies, in violation of **§ 2302(b)(8)**.

**3. Requested Action by OSC**

I respectfully request that OSC:

1. Investigate these additional allegations of **constructive demotion** and **investigative leave violations** as part of my existing complaint.

2. Consolidate these issues with my original May 28, 2025 PPP complaint.
3. Pursue corrective action restoring my approval authority and compliance responsibilities, removing improper adverse actions from my personnel record, and considering disciplinary action against responsible officials.

**Certification**

I certify that the information contained in this supplemental filing is true and correct to the best of my knowledge.

**/s/ Kevin D. Turnage**

Kevin D. Turnage, Complainant

[Date]

**Exhibit I – Proposed Corrective Action**

**OSC Case No. MA-25-004957 (Turnage, HHS/IHS)**

As part of the relief in this matter, I respectfully request that the U.S. Office of Special Counsel seek the following corrective actions:

**1. Restoration of Duties and Authority**

- Reinstate the **Policy & Procedure Nurse Consultant (Policy Officer)** as a required approver in the Policy Management Policy workflow.
- Confirm that my responsibilities match the compliance mandate of my Position Description, including authority to ensure organizational compliance with TJC, CMS, OSHA, and IHS standards.

**2. Compliance with Investigative Leave Statute**

- Correct Agency practices to comply with **5 U.S.C. § 6329b** by requiring written approval and periodic review for any investigative leave beyond 30 days.
- Remove any record of improperly extended investigative leave from my personnel file.

**3. Protection Against Retaliation**

- Ensure no further retaliatory actions are taken against me for my protected disclosures of May 21, 2025, and related activity.

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- OSC to consider recommending disciplinary action against officials who engaged in retaliation, including **CQO Clinton Pope**, who authored both the 2024 and 2025 Policy Management Policy revisions.

**4. Personnel Record Correction**

- Correct my official personnel record to reflect my original authority and responsibilities as Policy Officer.
- Remove any references or adverse documentation connected to retaliatory acts (e.g., unauthorized investigative leave, removal from workflow).

**5. Monitoring and Training**

- Require the Phoenix Area IHS and PIMC leadership to undergo **training on whistleblower protections and PPP compliance**, particularly regarding duty reassignment and investigative leave.
- Establish OSC monitoring for a defined period to ensure compliance.

**Respectfully submitted,**

**/s/ Kevin D. Turnage**

**Kevin D. Turnage**

Gilbert, AZ

September 24, 2025

Kevin Turnage

Digitally signed by Kevin Turnage
Date: 2025.09.24 22:14:30 -07'00'

CERTIFIED TRUE COPY
Printed on Sep 24 2025, 22:10